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EDITORIAL

Meaning as a Subject of Scientific Psychology

On the Launch of the Journal of Meaning-Centered Psychology

Eleanor M. Whitcombe, Sora Pazer and Samuel R. Ashford
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ABSTRACT

This issue marks the launch of the Journal of Meaning-Centered Psychology. The editorial outlines the journal's aims as a scholarly forum for the study of meaning in life, values, and existential concerns in psychology and psychotherapy. It starts from the observation that logotherapy, founded by Viktor E. Frankl, and contemporary empirical research on meaning have often developed in parallel rather than in dialogue. The journal seeks to bring these traditions into critical conversation without committing itself to any single therapeutic school. The editorial introduces the thematic focus of the first issue, which situates logotherapy within psychotherapy research: a review on the question of evidence, a theoretical paper on meaning in life as a transdiagnostic factor, a comparison of paradoxical intention with contemporary exposure-based treatments, and a methodological paper on the assessment of meaning in life. Finally, it sets out the journal's editorial principles, including a cautious and source-faithful reporting of empirical findings, a clear distinction between anthropological assumptions and empirically testable claims, and adherence to the citation guidelines of the American Psychological Association.

Keywords: meaning in life · logotherapy · psychotherapy research · editorial · philosophy of science

Aims of the Journal

The question of the meaning of one's own life is among the oldest themes of human self-reflection. As a subject of scientific psychology, however, it long played a marginal role. As early as the middle of the 20th century, Viktor E. Frankl put forward, with logotherapy, a psychotherapeutic approach that conceives of the striving for meaning as a fundamental human motivation (Frankl, 1946). Over the past two decades, empirical research on meaning has developed into an independent field of research that encompasses conceptual differentiations – for instance between coherence, purpose, and significance (Martela & Steger, 2016) – as well as the question of how prevalent the experience of meaning actually is in everyday life (Heintzelman & King, 2014).

The Journal of Meaning-Centered Psychology (Zeitschrift für Sinnzentrierte Psychologie, ZSP) seeks to take up these developments and to offer a forum for scientifically rigorous discussion. It regards itself neither as the organ of a single school nor as a place for the uncritical cultivation of tradition. Rather, its aim is to bring the anthropological and clinical insights of logotherapy and existential analysis into dialogue with the methodological awareness and the findings of contemporary psychotherapy research. This includes a willingness to examine cherished assumptions of one's own tradition just as critically as reductive operationalizations of a complex phenomenon.

That meaning can be a legitimate subject of empirical research was by no means always self-evident. For a long time, the question of meaning was considered too vague, too ideologically charged, or too much a matter for philosophy to be investigated psychologically. The development of reliable measurement instruments and differentiated theoretical models has changed this assessment. At the same time, there is a risk of reducing the phenomenon to what can easily be measured. A journal devoted to meaning must therefore accomplish both: methodological rigor and openness to the phenomenological complexity of its subject.

On the Focus of This Issue

The first issue is devoted to the focus "Logotherapy in the Context of Psychotherapy Research." The review by the Psychotherapy Research unit asks about the state of the evidence for logotherapeutic and meaning-centered interventions and shows that the answer depends substantially on which understanding of evidence is taken as a basis (Vos et al., 2015; Wampold & Imel, 2015). A theoretical contribution discusses the extent to which meaning in life can be understood as a transdiagnostic factor. A further contribution compares paradoxical intention with contemporary exposure procedures and works out similarities as well as differences in the assumed mechanisms of action. The issue concludes with a methods paper on established procedures for assessing meaning in life, which comparatively presents the strengths and limitations of the available instruments.

What the contributions share is an effort toward conceptual clarity. A construct such as "meaning," which is used in different ways in everyday language, philosophy, and psychology, in particular calls for careful specification before it can be adequately measured, investigated, or addressed therapeutically.

Editorial Principles

The ZSP publishes theoretical contributions, reviews, methods papers, contributions from practice and teaching, as well as book reviews. The editors attach importance to empirical findings being reported cautiously and faithfully to their sources, to limitations of the evidence being openly acknowledged, and to anthropological premises being distinguished from empirically testable statements. Didactic case examples are explicitly identified as constructed teaching vignettes. Citations follow the guidelines of the American Psychological Association (2020).

We thank all units of the Institute that have contributed to the success of this first issue, and we warmly invite our readers to accompany the discussion critically.

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REVIEW

Logotherapy and the Question of Evidence

A Narrative Review of the State of Research, Methodological Challenges, and Perspectives

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ABSTRACT

Logotherapy is one of the most established psychotherapeutic approaches of the twentieth century, yet it is rarely recognized as a distinct treatment within contemporary evidence-based psychotherapy. This narrative review examines how the current state of empirical research on logotherapeutic and meaning-centered interventions can be characterized and which methodological factors account for its comparatively narrow evidence base. First, it is argued that logotherapy comprises both an anthropological framework and a set of specific techniques, which complicates its operationalization as a testable treatment package. Second, central lines of evidence are summarized: the extensive, largely correlational research using meaning-in-life questionnaires, the meta-analytic integration of existential therapies, and randomized controlled trials of manualized meaning-centered programs in psycho-oncology. Overall, the findings suggest favorable effects of structured meaning-centered interventions but remain limited with respect to target populations, comparison conditions, and generalizability. Finally, the tension between the paradigm of disorder-specific efficacy trials and the contextual model of psychotherapy is discussed, together with the potential of process-oriented research that asks about mechanisms of change rather than school affiliation. The paper argues for a research agenda that respects anthropological assumptions while remaining methodologically compatible with mainstream psychotherapy research.

Keywords: logotherapy · evidence-based psychotherapy · meaning-centered psychotherapy · psychotherapy research · narrative review

1 Introduction

Since the 1990s, an understanding of evidence has become established in clinical psychology that judges psychotherapeutic procedures primarily by whether their efficacy for circumscribed disorders has been demonstrated in controlled studies (Chambless & Hollon, 1998). In the German-speaking countries, the work of Grawe et al. (1994) decisively shaped this development by programmatically seeking to lead psychotherapy "from confession to profession." Approaches that define themselves primarily through an image of the human being rather than through a disorder-specific treatment protocol easily come under pressure to justify themselves within this framework.

Logotherapy, founded by Viktor E. Frankl, is an instructive example of this. It has been received in numerous languages, has had a lasting influence on humanistic and existential psychotherapy, and is present in counseling, pastoral care, and palliative care. At the same time, it is hardly listed as an independent procedure in guidelines and textbooks of evidence-based psychotherapy. The present narrative review does not seek to resolve this discrepancy prematurely but rather to reconstruct its reasons. It draws on a selective choice of literature guided by its relevance to the research question and makes no claim to systematic completeness.

2 Logotherapy Between Anthropology and Treatment Procedure

Frankl himself understood logotherapy not as a procedure competing with other psychotherapies but as a supplement to them (Frankl, 1946). At its center is an image of the human being that assumes, in addition to the somatic and psychological dimensions, a spiritual, "noetic" dimension. From this dimension, Frankl derives freedom, responsibility, and the "will to meaning" as the primary motivation (Frankl, 1969). Accordingly, psychological suffering can arise, among

other things, when this will to meaning is frustrated; in this context, Frankl speaks of "noogenic neuroses" and of a widespread "existential vacuum."

In addition to this anthropological foundation, Frankl described specific techniques, in particular paradoxical intention for the treatment of anticipatory anxiety and obsessive-compulsive symptoms, and dereflection for excessive self-observation (Frankl, 1960). Later representatives have developed the methodology further, for instance in the sense of a modification of attitudes or of Socratic dialogue, and existential analysis has differentiated into an independent, more phenomenologically oriented procedure (Längle, 2013).

For research, this gives rise to a fundamental problem: it is not readily determinable what exactly is to be tested. If "logotherapy" is understood as an attitude that permeates any given treatment, it can hardly be delineated from a control condition. If, by contrast, it is reduced to individual techniques, it loses precisely those properties that its representatives consider central. This tension runs through the entire debate on evidence.

3 The State of Empirical Research

A first line of research concerns not the efficacy of logotherapy but the validity of its basic assumptions. Crumbaugh and Maholick (1964) had already developed the Purpose in Life Test, an instrument designed to make Frankl's concept of the existential vacuum psychometrically accessible. Subsequently, an extensive literature emerged that investigated associations between experienced meaning in life and mental health; an annotated bibliography of this work has been provided by Batthyány and Guttmann (2006). These studies are predominantly correlational in design and therefore permit no conclusions about the efficacy of therapeutic interventions, but they support the assumption that the experience of meaning is a clinically significant characteristic. More recent research on meaning has expanded these findings with independent instruments and models (Park, 2010; Steger et al., 2006).

A second line of research consists of controlled intervention studies. It should first be noted that studies on "classical" logotherapy in the narrower sense are comparatively rare and frequently show small samples, unclear manualization, or a lack of active control conditions. An important integrative contribution is the meta-analysis by Vos et al. (2015), which synthesized randomized controlled trials on various forms of existential therapy. The authors found indications of favorable effects, particularly for structured, explicitly meaning-centered interventions, whereas the evidence for other existential approaches was less clear-cut. At the same time, they emphasize the heterogeneity of the included studies and populations.

The methodologically best-investigated programs come from psycho-oncology. Meaning-Centered Group Psychotherapy, developed by Breitbart and colleagues and explicitly based on Frankl's work, was first examined in a randomized pilot study (Breitbart et al., 2010) and later in a larger randomized trial with an active control condition (Breitbart et al., 2015). In both studies, compared with supportive group psychotherapy, advantages emerged with regard to spiritual well-being and experienced meaning, as well as indications of a reduction in hopelessness and desire for hastened death. These findings are significant because they show that meaning-centered interventions can meet the standards of controlled research. However, they refer to a specific population and cannot readily be transferred to other clinical contexts.

4 Methodological Challenges

Several factors explain why the evidence base of logotherapy has remained narrow despite its long tradition. First, the logotherapeutic movement was for long stretches more firmly anchored in practice, teaching, and counseling than in university research. Second, the dual structure of anthropology and technique described above makes manualization more difficult. Third, the primary outcomes of logotherapy – experience of meaning, realization of values, attitude toward unalterable suffering – fit only to a limited extent with the symptom scales that usually serve as primary outcome criteria in disorder-specific efficacy studies.

In addition, there is a more fundamental question. Wampold and Imel (2015) contrasted the medical model of psychotherapy, which regards specific techniques as the main source of effects, with a contextual model that attaches considerable weight to the therapeutic relationship, plausible explanatory models, and the expectation of improvement. Grawe (1998), too, proposed a cross-school perspective with his common factors of change. From this point of view, the question of whether "logotherapy" as a brand-name procedure is effective would be less fruitful than the question of which of its elements contribute to change, and in what way. A review of the role of common factors, however, simultaneously

urges caution, since the evidence on these factors is also methodologically complex (Cuijpers et al., 2019).

A further problem concerns the comparison conditions. When meaning-centered interventions are compared with waiting lists or treatment as usual, nonspecific effects such as attention, structuring, and expectation cannot be separated from specific effects. Active control conditions, such as those used in the psycho-oncological studies (Breitbart et al., 2015), therefore represent methodological progress. Moreover, there is so far a near-total lack of studies that compare meaning-centered interventions directly with established disorder-specific procedures or that test their efficacy as a supplementary module within such procedures.

5 Perspectives for Process-Oriented Research

One way of turning the tension described into something productive lies in investigating mechanisms of change. Kazdin (2007) argued for systematically investigating mediators and mechanisms of therapeutic change instead of confining research to the comparison of treatment packages. For meaning-centered approaches, it seems natural to conceptualize changes in experienced meaning, in value orientation, or in the attitude toward unalterable life circumstances as possible mechanisms and to examine their temporal sequence in relation to symptom change.

This perspective fits into the debate on process-based psychotherapy, which places evidence-based processes rather than brand-name procedures at its center (Hofmann & Hayes, 2019). For logotherapy, such a framework offers the opportunity to translate its concepts into a common language with other procedures without abandoning its image of the human being. A prerequisite, however, is a precise operationalization of the postulated processes, a careful choice of instruments, and the willingness to take seriously results that do not conform to the hypotheses.

Concrete research questions arise from this perspective: Does an increase in experienced meaning precede the decrease in depressive symptoms in time, or does it follow it? Which patient groups benefit particularly from an explicit discussion of meaning and values? Under what conditions can the search for meaning, which in some studies is associated with distress (Steger et al., 2006), become therapeutically productive? Intensive longitudinal designs, systematic single-case analyses, and mediation models with repeated measurements are better suited to answering such questions than one-time pre–post comparisons.

6 Conclusion

The question of whether logotherapy is “evidence-based” cannot be answered with a simple yes or no. For the basic assumption that the experience of meaning is a clinically relevant characteristic, there is a broad, albeit predominantly correlational, body of findings. For manualized meaning-centered interventions in psycho-oncology, there are controlled studies with encouraging results. For logotherapy as a general psychotherapeutic procedure, by contrast, the evidence is limited. A sustainable research agenda should therefore aim less at legitimizing a school than at clarifying the mechanisms, indications, and limits of meaning-centered work.

For practice, this implies an attitude of well-founded modesty. Logotherapeutic concepts can enrich treatments, especially where questions of meaning, values, and coping with the unalterable are in the foreground. However, they should not be presented as a substitute for procedures whose efficacy for particular disorders is well documented. Such self-restraint, incidentally, corresponds to Frankl’s own understanding of logotherapy as a supplement to, not a replacement for, other psychotherapies (Frankl, 1946).

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THEORETICAL ARTICLE

Meaning in Life as a Transdiagnostic Factor

Conceptual Clarifications, Theoretical Models, and Open Questions

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ABSTRACT

Transdiagnostic approaches investigate psychological characteristics and processes that contribute to the onset and maintenance of psychological problems across diagnostic categories. This theoretical paper examines whether, and in what sense, meaning in life can be conceptualized as a transdiagnostic factor. Drawing on criteria discussed in the transdiagnostic literature, the paper first clarifies the concept of meaning in life, distinguishing between presence of meaning and search for meaning as well as between the components of coherence, purpose, and significance. Three models are then contrasted: meaning in life as a vulnerability or protective factor, as an outcome of successful meaning making after stressful events, and as an expression of overarching motivational structures. It is argued that a lack of experienced meaning should be understood less as a distinct mechanism than as a nodal point at which motivational, cognitive, and emotional processes converge. Finally, implications for assessment and psychotherapy are discussed, and methodological requirements for an empirical test of the transdiagnostic status of meaning in life are outlined, in particular longitudinal designs, the separation of conceptually overlapping constructs, and attention to cultural variability.

Keywords: meaning in life · transdiagnostic approach · psychopathology · meaning making · protective factors

1 Introduction

The categorical classification of mental disorders has proven indispensable for communication, health care, and research. At the same time, its limitations have long been known: high comorbidity rates, blurred boundaries between diagnoses, and the observation that similar psychological processes are at work across different disorders have led to a transdiagnostic perspective. In an influential monograph, Harvey et al. (2004) showed that processes such as selective attention, repetitive negative thinking, or avoidance behavior occur across numerous disorders. At the structural level, it has also been discussed whether a general psychopathology factor can be identified (Caspi et al., 2014).

Against this background, it is natural to ask whether meaning in life – or its absence – can also be understood as a transdiagnostic factor. Logotherapy anticipated such an assumption in a certain sense by describing the frustration of the will to meaning as a condition that can manifest itself in different symptoms (Frankl, 1969). The present contribution examines this intuition in the light of current theoretical criteria. It is intended as a conceptual analysis and not as a summary of original empirical data.

2 Criteria for Transdiagnostic Constructs

Nolen-Hoeksema and Watkins (2011) proposed a heuristic by which transdiagnostic models can be specified. They distinguish distal risk factors that increase vulnerability, proximal processes that are directly involved in symptom formation, and moderating conditions that explain why a common factor leads to different disorders in different individuals. Accordingly, a construct does not deserve the label “transdiagnostic” merely because it correlates with several disorders. What is required, rather, is a theoretically grounded model of how it affects the development and maintenance of symptoms.

For meaning in life, this gives rise to a twofold task. First, it must be clarified what exactly is understood by meaning in life. Second, it must be determined at which level of the model – as a distal factor, as a proximal process, or as a moderator – it is to be

located. The two questions are not always kept sufficiently apart in the literature.

3 Conceptual Specification

With the distinction between the presence of meaning and the search for meaning, Steger et al. (2006) introduced a differentiation with far-reaching consequences for empirical research. While the experienced presence of meaning is generally associated with well-being, the relationship between the search for meaning and mental health is more complex and apparently context-dependent. Martela and Steger (2016) further distinguished three facets of the experience of meaning: coherence in the sense of the comprehensibility of one's own life, purpose in the sense of a motivating orientation toward valued goals, and significance in the sense of experiencing one's own life as valuable and as mattering.

Schnell (2009) has also shown that a distinction should be made between the sources from which people derive meaning and the global experience of meaning. This distinction is clinically significant: a lack of experienced meaning can be due to the loss of individual sources of meaning, to a low diversity of such sources, or to a lack of integration into an overarching self-understanding. Furthermore, Schnell (2020) has drawn attention to the state of "existential indifference," in which meaning is neither experienced nor sought – a state that differs clearly from an acute crisis of meaning.

For the transdiagnostic question, it follows that "meaning in life" is not a unitary construct. Anyone who speaks of meaning as a transdiagnostic factor must specify which facet is meant, since coherence, purpose, and significance are potentially linked to different psychopathological processes.

In addition, the experience of meaning and the search for meaning should not be understood as opposite poles of a single dimension (Steger et al., 2006). A person may experience their life as meaningful and at the same time search for deeper meaning; conversely, someone may experience little meaning without actively searching for it. These constellations presumably have different clinical significance, which must be taken into account when formulating transdiagnostic hypotheses.

4 Three Model Conceptions

A first model understands experienced meaning as a protective factor and its lack as a vulnerability factor. This view is related to salutogenic considerations such as those formulated by Antonovsky (1987) with the concept of the sense of coherence, in which meaningfulness occupies a central position as the motivational component. In this model, meaning in life would be a distal factor in the sense of Nolen-Hoeksema and Watkins (2011) that influences the probability of various disorders without determining their specific form.

A second model emphasizes the process dimension. In her meaning-making model, Park (2010) describes how stressful events create a discrepancy between global meaning structures and the situational appraisal of an event, which can be reduced through processes of meaning making. Within this framework, psychological complaints could be understood as an expression of unsuccessful meaning making. Here, meaning is not primarily a stable trait but the result of ongoing processing that may be proximally involved in the maintenance of symptoms.

Finally, a third model interprets the experience of meaning as an expression of overarching motivational structures. Baumeister (1991) described needs for meaning relating to purpose, values, efficacy, and self-worth. From this perspective, a lack of meaning points to a disturbance of motivational orientation, which may manifest itself, for example, in reduced drive, avoidance behavior, or compensatory strategies. Yalom (1980), too, described meaninglessness as one of the ultimate concerns of existence, with which people deal in very different, sometimes dysfunctional ways.

Logotherapy cannot be clearly assigned to any of these models. Frankl's concept of noogenic neurosis describes complaints that arise primarily from an existential conflict or from existential frustration (Frankl, 1969). It thereby assumes a specific etiology that is supposed to differ from psychogenic and somatogenic disorders. At the same time, Frankl emphasizes that an existential vacuum can also exist without pathological significance and can manifest itself in different forms. From a transdiagnostic point of view, this dual function is remarkable because it conceptualizes the frustration of meaning both as a specific cause and as a nonspecific background factor.

5 Meaning as a Nodal Point: An Integrative Interpretation

The three models are not mutually exclusive. There is much to be said for understanding a lack of experienced meaning less as an independent, isolable mechanism than as a nodal point at which motivational, cognitive, and emotional processes converge. Findings according to which positive affect

influences the momentary experience of meaning (King et al., 2006) illustrate that the experience of meaning is not independent of a person's affective state. Conversely, it is plausible that a stable value orientation can buffer affective crises.

Such a nodal-point interpretation has consequences for research. It warns against treating the experience of meaning as a mere reflection of depressive symptoms, as well as against inflating it into an all-encompassing explanatory principle. Heintzelman and King (2014) have also pointed out that most people describe their lives as fairly meaningful. A clinically relevant lack of meaning is therefore the exception rather than the rule, which is significant for determining cut-off values and risk groups.

A further aspect concerns the direction of the associations. Cross-sectional correlations between a low experience of meaning and psychological complaints are compatible with several causal assumptions: a lack of meaning may promote complaints, complaints may impair the experience of meaning, or both may be due to common third factors. A transdiagnostic model must explicitly distinguish these possibilities and must not prematurely infer directions of effect from associations.

6 Implications and Open Questions

For diagnostic assessment, the analysis suggests assessing the experience of meaning in a facet-specific and cross-disorder manner rather than merely recording it as a concomitant variable. For psychotherapy, the question arises whether meaning-centered interventions can be used as supplementary modules in different treatment contexts. This would correspond to Frankl's self-understanding of logotherapy as a supplement to other procedures.

An empirical test of the transdiagnostic status of meaning in life would require, in particular, longitudinal designs that capture temporal sequences between the experience of meaning and symptom development, as well as a careful separation of conceptually overlapping constructs such as hopelessness, well-being, or self-efficacy, and consideration of cultural variability in the significance of meaning. As long as these requirements are not met, talk of meaning in life as a transdiagnostic factor should be understood as a heuristically fruitful hypothesis, not as an established finding.

For therapeutic practice, it should also be borne in mind that addressing meaning is not appropriate at every point in a treatment. In acute crises, with severe depressive symptoms, or with pronounced reduction in drive, the explicit question of the meaning of one's own life can be overwhelming or intensify feelings of failure. A transdiagnostic understanding of meaning therefore does not argue for an undifferentiated application of meaning-centered techniques but for careful determination of the indication that takes into account the person's condition, their resources, and the course of treatment.

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THEORETICAL ARTICLE

Paradoxical Intention and Modern Exposure Procedures

A Theoretical Comparison of Assumed Mechanisms, Procedure, and Limitations

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ABSTRACT

Paradoxical intention is among the best-known techniques of logotherapy. Viktor E. Frankl described it as early as the period between the 1930s and the 1950s as a method for interrupting anticipatory anxiety in phobias and obsessive-compulsive symptoms. This paper compares paradoxical intention at a theoretical level with contemporary exposure-based procedures in cognitive behavioral therapy, in particular with emotional processing theory and the inhibitory learning approach. First, substantial similarities are identified: both approaches aim at relinquishing avoidance and control behaviors, encourage confrontation with feared experiences, and enable corrective learning. At the same time, there are differences in their anthropological rationale, in the role of humor and self-distancing, and in how the procedure is structured. The paper then discusses to what extent concepts such as ironic processes of mental control, metacognitive models, and cognitive defusion may serve as bridges between the two traditions. It concludes with a discussion of the limits of indication and with suggestions for a more precise theoretical placement of paradoxical intention within current psychotherapy research, without abandoning its distinct existential-analytic foundation.

Keywords: paradoxical intention · exposure therapy · inhibitory learning · self-distancing · anxiety disorders

1 Introduction

Among the techniques of logotherapy, paradoxical intention has achieved the widest dissemination beyond its own school. In a much-cited paper, Frankl (1960) described it as a procedure in which patients are instructed to wish for, or deliberately bring about, precisely what they fear instead of strenuously avoiding it. In his account of the theory of neuroses, he systematically embedded this approach in a model of anticipatory anxiety (Frankl, 1956).

Since then, the treatment of anxiety and obsessive-compulsive disorders has developed considerably. Exposure-based procedures are today among the best-researched psychotherapeutic interventions of all (Abramowitz et al., 2019). It therefore seems natural to compare paradoxical intention with these procedures. The present contribution does so at the theoretical level. It asks which assumptions about mechanisms of action underlie the two approaches, where they converge, and where they differ.

The comparison is also worthwhile because paradoxical intention is occasionally referred to in the literature as a forerunner of behavioral techniques. Such a classification is historically understandable but does only partial justice to Frankl's self-understanding. It is therefore necessary to take seriously both the functional kinship and the independent rationale.

2 Paradoxical Intention as Understood in Logotherapy

Frankl's starting point is a circular model: a symptom triggers the fear that it might recur; this anticipatory anxiety in turn promotes the renewed occurrence of the symptom, which further reinforces the fear (Frankl, 1956). According to Frankl, typical patterns of reaction to this vicious circle are flight from fear in phobic complaints and fighting against the obsessive idea in obsessive-compulsive symptoms. Both patterns contribute to the entrenchment of the problem.

Paradoxical intention intervenes at this point. It asks the affected person to wish consciously for what they fear – for instance blushing, trembling, or sweating – and, where possible, to do so in an exaggerated, humorous form. Frankl (1960) emphasized that this technique is based on the specifically human capacity for self-distancing, which he attributed to the noetic dimension. Humor is not a decorative addition here but a central means of gaining distance from the symptom and depriving the fear of its power.

It is noteworthy that Frankl did not understand paradoxical intention as a mere technique. It was intended to enable the person not to identify with their fear but to adopt a stance toward it. This expresses the basic anthropological assumption of logotherapy that human beings are not entirely at the mercy of their psychological conditions but can take a stand toward them.

Frankl also distinguished paradoxical intention from dereflection. While paradoxical intention targets anticipatory anxiety, dereflection addresses excessive self-observation, which Frankl called hyperreflection and described, for example, in sleep disorders and sexual dysfunctions (Frankl, 1956). Common to both procedures is the assumption that a certain form of strained attention to oneself maintains the very problem it seeks to solve.

3 Exposure Procedures: From Emotional Processing to Inhibitory Learning

The theoretical rationale for exposure has changed over the past decades. With emotional processing theory, Foa and Kozak (1986) formulated a model according to which fear is represented in memory structures that can be modified through activation and the incorporation of corrective information. In this model, habituation within and between practice sessions was regarded as an important indicator of successful processing.

Craske et al. (2014), by contrast, proposed an inhibitory learning model. According to this model, the original fear association is not erased by exposure but supplemented by newly learned, inhibitory associations that compete with the original one. For practice, it follows that the focus is less on reducing fear during the exercise than on violating expectancies, varying practice contexts, and refraining from safety behaviors. Tolerating fear thus becomes a goal in itself.

Also significant for practice is that modern exposure procedures are usually based on careful case conceptualization and a convincing rationale that makes it comprehensible to patients why confronting fear is worthwhile (Abramowitz et al., 2019). Here, too, a parallel emerges: Frankl attached importance to ensuring that those affected understand the mechanism of anticipatory anxiety before they apply paradoxical intention.

4 Convergences

From the perspective of inhibitory learning, paradoxical intention appears in a new light. By wishing for what they fear, patients give up avoidance and control strategies and expose themselves to the feared experience. The expectation that the symptom will escalate catastrophically is frequently not confirmed in the process. Viewed functionally, paradoxical intention thus contains central elements of an exposure exercise: confrontation, abandonment of safety behaviors, and expectancy violation.

Research on thought control offers a further bridge. Wegner (1994) showed that attempting to suppress certain thoughts can, under certain conditions, lead precisely to their increased occurrence. This concept of ironic processes provides an experimentally grounded explanation for the observation, described clinically by Frankl, that fighting a symptom can intensify it. The metacognitive model of Wells (2009), which focuses on problematic beliefs about the controllability and dangerousness of one's own thoughts, also shows points of contact here.

Finally, there are parallels with cognitive defusion in Acceptance and Commitment Therapy, in which thoughts are regarded as passing mental events rather than being treated as literal truths (Hayes et al., 2012). Some defusion exercises that work with exaggeration, repetition, or playful alienation resemble, in their structure, the humorous distancing that Frankl described.

5 Differences

Despite these convergences, it would be reductive to interpret paradoxical intention simply as an early form of exposure. A first difference concerns the rationale: while exposure procedures are grounded in the psychology of learning and memory, Frankl invokes an anthropological capacity for self-distancing. A second difference lies in the degree of structuring. Modern exposure protocols are usually carefully planned, with hierarchies, expectancy checks, and debriefings. Paradoxical intention, by contrast, was often conveyed as a compact attitude to be applied in everyday life.

A third difference concerns the role of humor. In exposure procedures, humor is at most a facilitative contextual factor, whereas in paradoxical intention it is constitutive. Whether and to what extent humor itself contributes to efficacy is an open empirical question. In addition, paradoxical interventions have been studied in the literature under very different names and procedural formats. An early meta-analysis of paradoxical interventions (Shoham-Salomon & Rosenthal, 1987) does point to a certain efficacy but at the same time indicates considerable heterogeneity, so that conclusions about the logotherapeutic technique in the narrower sense can be drawn only with caution.

A fourth difference concerns the goal. Exposure procedures are directed primarily at reducing fear and avoidance or at improving fear tolerance. For Frankl, by contrast, overcoming anticipatory anxiety was not an end in itself but a precondition for the person being able to turn once again to the tasks and values of their life. In this respect, paradoxical intention is close to Acceptance and Commitment Therapy, which likewise does not regard fear reduction as the primary goal but places value-oriented action at the center (Hayes et al., 2012).

6 Limits of Indication and Outlook

Frankl (1956) himself pointed out that paradoxical intention is not indicated for all clinical presentations. In depressive states with suicidality, for example, an invitation to wish for what one fears would obviously be contraindicated. In the case of obsessive thoughts with aggressive content, too, careful psychoeducation is required so that the technique is not experienced as trivialization. A sound working alliance and a clear rationale for the intervention are therefore indispensable.

For future research, the inhibitory learning model offers a framework in which hypotheses about how paradoxical intention works can be formulated precisely, for instance regarding the role of expectancy violation or the contribution of humor and self-distancing. Such an integration would not deprive paradoxical intention of its existential-analytic rationale but would supplement it with a compatible mechanism perspective.

Methodologically, component analyses lend themselves to testing such hypotheses, in which, for example, an exposure condition with and without a humorous-paradoxical instruction is compared. In this way, it could be clarified whether the paradoxical formulation makes an independent contribution beyond the confrontation. It would also be worth investigating whether people with a pronounced tendency toward thought suppression or with metacognitive beliefs about the dangerousness of their own thoughts benefit particularly from a paradoxical stance (Wegner, 1994; Wells, 2009).

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METHODS ARTICLE

Measuring Meaning in Life

The Purpose in Life Test, the Meaning in Life Questionnaire, and LeBe/SoMe in Conceptual and Methodological Comparison

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ABSTRACT

Empirical research on meaning in life is inseparably linked to the development of suitable measurement instruments. This methodological paper compares three families of instruments that are particularly widely used in research and practice: the Purpose in Life Test, developed within the tradition of logotherapy; the Meaning in Life Questionnaire with its distinction between presence of meaning and search for meaning; and the Sources of Meaning and Meaning in Life Questionnaire (SoMe) and its German counterpart LeBe, which additionally assess sources of meaning. The Existence Scale is considered as an instrument rooted in the existential-analytic tradition. The instruments are described with respect to their theoretical foundations, structure, construct delineation, and typical fields of application. The central focus is on construct validity: the paper discusses to what extent the instruments distinguish experienced meaning from related constructs such as well-being, depressive symptoms, or life satisfaction, and which facets of meaning each of them captures. Finally, recommendations for the selection of instruments in research, assessment, and evaluation are formulated, including orientation toward the research question, the combination of global and source-specific measures, and a cautious interpretation of sum scores in individual assessment.

Keywords: meaning in life · psychological assessment · construct validity · questionnaires · measurement

1 Introduction

A psychological construct can only be researched as well as it can be measured. This applies to meaning in life to a particular degree, because the term is used with very different meanings in everyday language, philosophy, and psychotherapy. The choice of a measurement instrument is therefore never merely a technical decision but always implies a theoretical commitment as to what is understood by meaning.

The present contribution presents selected, frequently used procedures in comparison. It does not aim at a complete overview but seeks to work out the conceptual differences between the instruments and to derive recommendations for practice from them. It is guided by an understanding of validity as a unitary concept relating to the interpretation of test scores (American Educational Research Association et al., 2014; Messick, 1995).

In addition, research on meaning in life relies to a high degree on self-report procedures. Alternative approaches such as qualitative interviews, narrative procedures, or everyday assessments with repeated brief surveys complement the methodological repertoire but are considered only peripherally in what follows.

2 The Purpose in Life Test

Crumbaugh and Maholick (1964) developed the Purpose in Life Test (PIL) in order to capture Frankl's concept of noogenic neurosis and the existential vacuum psychometrically. In its best-known form, the instrument consists of 20 statements answered on a seven-point scale, each with individually worded anchors; supplementary qualitative parts with sentence completions and a free essay are used less frequently in research. For decades, the PIL was the most frequently used meaning instrument, and it forms the basis of a large part of logotherapeutically oriented research.

At the same time, the PIL has been widely criticized. In an early analysis, Dyck (1987) pointed to conceptual and psychometric problems, in particular

the heterogeneity of item content and its overlap with constructs such as depressiveness and life satisfaction. Some items capture mood or zest for life rather than meaning in the narrower sense. This criticism concerns discriminant validity and is significant for the interpretation of associations with symptom measures. With the short form PIL-SF, Schulenberg et al. (2011) presented an economical four-item variant that focuses more closely on the core of the construct.

A further problem lies in the wording of some items, which implicitly contain value judgments, for instance with regard to life goals or attitudes toward death. Such formulations may be understood differently depending on culture and worldview. For the interpretation of group differences, measurement invariance must therefore be tested before means are compared between groups.

3 The Meaning in Life Questionnaire

Steger et al. (2006) developed the Meaning in Life Questionnaire (MLQ) with the explicit aim of overcoming weaknesses of earlier procedures. The instrument comprises ten items assigned to two subscales: the presence of meaning and the search for meaning. The authors placed particular emphasis on distinguishing the construct from related constructs and reported evidence of convergent and discriminant validity in their validation studies.

The strength of the MLQ lies in its brevity, its clear structure, and the conceptually significant separation of presence and search. One limitation is that the items are worded in comparatively global terms and provide no information about where a person derives meaning from or which facet of the experience of meaning is in the foreground. The distinction between coherence, purpose, and significance proposed by Martela and Steger (2016) is not represented in a differentiated way by the MLQ.

For the interpretation of the Search subscale, it is important that the search for meaning must not be understood per se as a deficit. Steger et al. (2006) conceptualized presence and search as relatively independent dimensions. Depending on the phase of life and the context, the search for meaning can be an expression of openness and development or of uncertainty and distress. It therefore becomes diagnostically informative only in combination with the degree of experienced meaning.

4 LeBe and SoMe: Experience of Meaning and Sources of Meaning

With the Questionnaire on Sources of Meaning and Meaning in Life (Fragebogen zu Lebensbedeutungen und Lebenssinn, LeBe), Schnell and Becker (2007) presented a procedure that goes beyond the global experience of meaning. In addition to scales on meaningfulness and crisis of meaning, the instrument assesses a larger number of sources of meaning, that is, sources from which people can derive meaning. These are assigned to superordinate dimensions ranging from vertical and horizontal self-transcendence through self-actualization and order to well-being and relatedness. The English-language version, SoMe, was described by Schnell (2009).

The particular achievement of this approach lies in the separation between the degree of the experience of meaning and its sources. This makes accessible research questions that cannot be investigated with global measures, for instance whether the experience of meaning depends more on the diversity or on the intensity of individual sources of meaning. In addition, the separate assessment of meaningfulness and crisis of meaning allows the identification of people who neither experience meaning nor suffer from its absence (Schnell, 2020). The greater length of the instrument must, however, be taken into account when planning studies and diagnostic assessments.

For practice, it should be emphasized that the source-related profile can offer starting points for counseling and therapy, for instance when it becomes apparent that a person's experience of meaning rests on very few sources. Such profiles should not, however, be read normatively: there is no composition of sources of meaning that is equally right for all people.

5 The Existence Scale

The Existence Scale (Existenzskala, ESK) by Längle et al. (2000) stems from the tradition of existential analysis. It does not primarily capture the experience of meaning but rather personal capacities that, according to the existential-analytic understanding, are prerequisites for a meaningful way of life: self-distancing, self-transcendence, freedom, and responsibility. It thereby operationalizes central concepts of Frankl's anthropology and is particularly suitable for research questions aimed at existential resources and their change in the therapeutic process.

For evaluation studies of logotherapeutic and existential-analytic interventions, the Existence Scale is of particular interest because it captures capacities

that, according to the underlying theory, are supposed to be fostered by treatment. It is therefore potentially suitable as a process measure. At the same time, it should be noted that the scale is closely tied to a particular theoretical model; its results can therefore be interpreted mainly within the framework of that model (Längle et al., 2000).

6 Construct Validity as the Guiding Question

Cronbach and Meehl (1955) already emphasized that construct validity cannot be demonstrated by a single coefficient but can only be made plausible within a nomological network. For the measurement of meaning in life, this means that instruments should not only be associated with well-being and mental health but should at the same time be distinguishable from these constructs. An instrument that largely captures the experience of meaning as the opposite of depressed mood is hardly suitable for testing the hypothesis that meaning acts as an independent protective factor.

The distinction from eudaimonic well-being is also significant. Ryff (1989) conceptualized purpose in life as one of several dimensions of psychological well-being. Anyone who assesses the experience of meaning and well-being simultaneously should therefore check whether the instruments used contain items with overlapping content. Otherwise, associations may be partly due to item overlap rather than to substantive relationships.

A further challenge concerns the distinction between state and trait aspects. The experience of meaning may be relatively stable over time but is also subject to situational fluctuations, as studies on its association with positive affect suggest (King et al., 2006). For measuring change in therapy and counseling, it must therefore be examined whether an instrument is sufficiently sensitive to change; for interpreting individual change, indices such as the standard error of measurement should be used.

7 Recommendations for Choosing an Instrument

Several recommendations can be derived from the comparison. First, the choice of an instrument should proceed from the research question: for economical assessments of the global experience of meaning, the MLQ is a good choice; for the investigation of sources of meaning and crises of meaning, the LeBe or SoMe; for the assessment of existential capacities, the Existence Scale. The PIL remains significant for comparability with older research but should be interpreted with awareness of its overlap with related constructs.

Second, in many cases the combination of a global measure with a source- or facet-related measure appears advisable. Third, restraint is called for in individual diagnostic assessment: questionnaire scores on meaning in life can supplement, but not replace, a clinical conversation about values, goals, and life circumstances. Finally, when using translated instruments, attention must be paid to validated language versions and to the appropriateness of the norms for the respective target group.

Finally, reports should state which version of an instrument, which response scale, and which scoring rules were used. In the case of the PIL in particular, different versions and translations exist, which makes the comparability of findings more difficult. Transparent information facilitates replications and meta-analytic integration and meets the requirements for comprehensible test use (American Educational Research Association et al., 2014).

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